HEALTH BEACON

CMSRN EXAM PREP

Practice Exam 01 - Nursing Teamwork and Collaboration

26 scored practice questions | Printable Practice Exam

Instructions	Select the single best answer for each question. An answer key with explanations begins after the question section.

PART A - QUESTIONS

1.

Scenario

An RN delegates hallway ambulation to a trained unlicensed assistive personnel (UAP) for a patient who is 1 day postoperative after a colectomy. Before standing, the patient tells the UAP, "I suddenly feel lightheaded." The UAP reports this to the RN.

Which response by the RN best demonstrates appropriate supervision of the delegated task?

A) Ask the UAP to obtain a wheelchair and complete the ambulation later

B) Assess the patient and determine whether ambulation remains appropriate

C) Tell the UAP to measure the walking distance if the symptoms resolve

D) Reassign the ambulation to another UAP with more experience

2.

Scenario

A medical-surgical unit is considering bedside shift report. Some nurses expect it to improve communication, while others are concerned about interruptions and privacy. The nurse manager wants staff participation without delaying a needed safety improvement.

Which leadership approach is most likely to support a sustainable change?

A) Announce a mandatory implementation date and address objections after launch

B) Allow each nurse to choose whether to use bedside or nurses' station report

C) Post evidence supporting bedside report and wait for resistance to decrease

D) Engage staff in designing a limited trial, define outcome measures, and revise the process from feedback

3.

Scenario

A preceptor observes an orientee give discharge instructions rapidly, ask "Do you understand?" and leave before the patient demonstrates wound care. The patient later asks the preceptor to repeat the instructions.

Which preceptor response best promotes the orientee's reflective learning?

A) Ask the orientee to analyze the interaction, identify missed cues, and plan a teach-back approach

B) Repeat the teaching personally and postpone discussion until the final orientation evaluation

C) Give the orientee a written script and require the exact wording for every discharge

D) Document that the orientee failed communication competency after this single event

4.

Scenario

A unit peer-review group examines a patient fall that occurred after a toileting plan was not updated when the patient's mobility declined. The purpose of the review is professional learning and practice improvement rather than discipline.

Which method should the group use?

A) Discuss the nurse's prior reputation before reviewing the event documentation

B) Share the nurse's identity so colleagues can provide informal performance history

C) Compare objective actions with practice standards and identify individual and system learning needs

D) Limit the review to whether the nurse completed the incident report on time

5.

Scenario

Facility policy permits an LPN to administer routine oral medications, reinforce established teaching, and collect focused data for stable patients. The policy does not permit an LPN to perform an initial assessment or give IV push medications. The RN is making assignments.

Which patient is most appropriate to assign to the LPN?

A) A stable patient with heart failure who needs routine oral medications and reinforcement of low-sodium teaching

B) A newly admitted patient with fever who needs an initial assessment and care plan

C) A patient with sudden chest pressure who requires immediate assessment and an IV push medication

D) A patient returning from surgery whose condition is changing and requires frequent nursing judgments

6.

Scenario

Two nurses call out for the night shift. The unit has several high-acuity patients, and the hospital has a qualified float nurse available. Agency staffing would cost more and arrive later.

Which action by the charge nurse best balances patient safety and fiscal stewardship?

A) Divide all patients equally among the nurses already scheduled

B) Match assignments to acuity and competencies, use the qualified float nurse, and reassess workload during the shift

C) Close beds immediately without reviewing current discharges or available staff

D) Replace both absent nurses with agency nurses to preserve the usual staffing pattern

7.

Scenario

An RN asks a UAP to measure and record urine output for a stable patient after abdominal surgery and to report output below 30 mL in an hour. The UAP reports 15 mL each hour for the past 2 hours.

What should the RN do next?

A) Ask the UAP to encourage oral fluids and recheck output in 2 hours

B) Enter the values in the record and wait for the next routine assessment

C) Assess the patient, validate relevant findings, and determine whether escalation is needed

D) Tell the UAP to irrigate the urinary catheter to check for obstruction

8.

Scenario

A unit is implementing barcode medication administration. Staff members can explain why the change is needed and say they want it to succeed, but several cannot demonstrate the scanning and override workflow. The manager is using the ADKAR change framework.

Which intervention addresses the team's current change need?

A) Provide hands-on training and practice with feedback before independent use

B) Present additional data about the frequency of medication errors

C) Recognize staff who have sustained the new process for several months

D) Survey staff about whether they personally support safer medication administration

9.

What should a medical-surgical RN do before independently using an unfamiliar low-frequency clinical device?

A) Review the manufacturer's quick guide while using the device on a patient

B) Ask a coworker to remain nearby during the first independent use

C) Rely on prior experience with a device that performs a similar function

D) Complete the required education and demonstrate competency according to organizational policy

10.

Scenario

Two nurses disagree about an assignment and begin criticizing each other at the nurses' station where patients and visitors can hear. Patient care has not yet been delayed.

How should the charge nurse respond?

A) Send both nurses home and divide their patients among the remaining staff

B) Ignore the disagreement unless a patient-care task is missed

C) Move the discussion to a private area, clarify the patient-care facts, and facilitate a respectful solution

D) Decide which nurse is at fault and announce the decision to the team

11.

Scenario

A nurse with 18 months of medical-surgical experience asks a mentor for help preparing for a future charge nurse role. The nurse has not yet identified specific development needs.

Which mentor action is most appropriate at the start of the relationship?

A) Arrange a charge assignment next week to test the nurse's readiness

B) Help the nurse define goals, assess strengths and gaps, and schedule progress reviews

C) Select leadership courses for the nurse based on the mentor's own career path

D) Promise to recommend the nurse for the next opening if unit performance remains strong

12.

Scenario

An RN is coordinating care with an experienced, competency-validated UAP. One patient is newly confused, one has chest pressure, one needs discharge teaching, and one stable patient needs assistance with bathing.

Which task should the RN delegate to the UAP?

A) Determine possible causes of the new confusion

B) Obtain the initial history for the patient with chest pressure

C) Evaluate whether the discharge teaching was effective

D) Assist the stable patient with bathing and report any skin concerns

13.

Scenario

After an industrial chemical release, the hospital activates its emergency operations plan. A medical-surgical RN assigned near the receiving area sees a self-referred patient who may be contaminated walking toward the clean treatment zone.

Which action best supports the emergency procedure?

A) Escort the patient directly to an available inpatient room

B) Ask registration staff to complete the admission before further movement

C) Stop the patient from entering the clean zone and route the patient through the designated decontamination process

D) Give the patient clean clothing and allow entry if no symptoms are present

14.

Scenario

An RN is leaving the unit for a meal break. A scheduled IV antibiotic will finish during the break, and one patient's pain must be reassessed after an intervention. Another RN has agreed to provide coverage.

Which action best transfers responsibility for these activities during the break?

A) Give a concise handoff naming the required follow-up and confirm that the covering RN accepts it

B) Tell the patients to use the call light if either activity is delayed

C) Ask a UAP to remind the covering RN when each activity is due

D) Document the planned activities before leaving and assume the record is sufficient notice

15.

Scenario

An orientee administers medications accurately but consistently falls behind because supplies are gathered separately for each room and documentation is postponed until late in the pass. The preceptor wants to coach without compromising safety.

Which approach is best?

A) Reduce the orientee's patient load permanently and avoid discussing efficiency

B) Tell the orientee to work faster and compare completion times with peers

C) Complete the late medications for the orientee without reviewing the workflow

D) Observe the workflow, give behavior-specific feedback, and set progressive time-management goals

16.

Scenario

A nurse selects the wrong concentration of a look-alike medication but notices the error during the independent check, before administration. The two concentrations are stored beside each other, and other nurses report similar confusion.

Which response by the manager best reflects a just culture approach?

A) Discipline the nurse because selecting the wrong product is never acceptable

B) Take no action because the medication did not reach the patient

C) Examine the system hazards and the nurse's behavioral choices, then respond proportionately

D) Remove all independent medication checks because they did not prevent product selection

17.

Scenario

A nurse identifies difficulty interpreting trends in renal function when caring for patients receiving potentially nephrotoxic medications. The nurse and educator are creating an individual development plan.

Which plan element will best support competency development?

A) Attend any available hospital lecture and consider the need resolved

B) Set a measurable objective, select relevant learning and practice activities, and evaluate performance

C) Read several articles without defining how learning will be applied

D) Wait for the annual evaluation to determine whether improvement occurred

18.

Scenario

An RN delegates meal setup and feeding assistance for a stable patient to a trained UAP. The RN instructs the UAP to stop and report coughing, a wet voice, or difficulty swallowing. Soon after feeding begins, the UAP reports repeated coughing.

What should the RN direct next?

A) Offer smaller bites and continue feeding more slowly

B) Replace solid food with thin liquids and observe the response

C) Document the patient's poor appetite and try again at the next meal

D) Stop oral intake and assess the patient before feeding resumes

19.

Scenario

A unit changes from total patient care to team nursing. During the first week, RNs and assistive personnel report duplicated tasks, missed updates, and uncertainty about who follows up abnormal findings.

Which action by the nurse manager best addresses the care-delivery problem?

A) Clarify roles and accountability, then use brief team huddles to coordinate and update the plan

B) Return immediately to total patient care before evaluating the new workflow

C) Allow each team to define roles informally as the shift progresses

D) Assign all communication responsibilities to the most experienced UAP

20.

Scenario

During a compliance audit, the manager discovers that a nurse's annual competency validation for a specialized central-line procedure expired last month. The procedure is scheduled for a patient later today, and another validated nurse is available.

What is the manager's best response?

A) Permit the nurse to perform the procedure while a coworker watches

B) Assign the procedure to the validated nurse and arrange competency revalidation before future independent performance

C) Cancel all central-line procedures until every nurse on the unit is revalidated

D) Backdate the validation because the nurse performed the procedure successfully last year

21.

Scenario

A charge nurse notes that use of a costly specialty dressing on the unit is well above budget. The dressing is clinically indicated for some wounds, but supply records suggest it may also be used routinely when lower-cost products would meet the ordered plan of care.

Which supervisory action best supports fiscal efficiency without compromising care?

A) Lock up the dressing and require manager approval for every use

B) Replace the dressing with the least expensive product for all patients

C) Review utilization against indications, address knowledge gaps, and standardize appropriate access

D) Ask staff to bring unused dressings from patients' rooms back to central supply

22.

Scenario

A nurse calls a clinician about a patient whose blood pressure and mental status have worsened. The clinician dismisses the concern without reviewing the new findings. The nurse restates the concern, but no evaluation or order is provided.

Which leadership behavior should the nurse use next?

A) Wait for the next scheduled vital signs to avoid creating conflict

B) Document the call and transfer responsibility for the outcome to the clinician

C) Ask another staff nurse to decide whether the findings are serious

D) State that the situation is a safety concern and activate the chain of command if it remains unresolved

23.

Which use of a professional nursing organization most directly supports ongoing clinical competence?

A) Apply a relevant practice update, evaluate its effect on care, and share learning with colleagues

B) Join the organization primarily to list membership on an annual evaluation

C) Attend a networking event without identifying a practice or development goal

D) Rely on membership materials instead of organizational policies and competency requirements

24.

Scenario

Severe flooding causes the hospital to activate its incident command structure. A unit charge nurse receives conflicting requests from three different departments for staff, beds, and supplies. The charge nurse has an assigned incident-command supervisor.

How should the charge nurse proceed?

A) Follow the request from the highest-ranking department leader

B) Report the competing needs through the assigned chain and obtain a single prioritized direction

C) Divide the unit's resources equally among the requesting departments

D) Delay all responses until the hospital returns to its routine structure

25.

Scenario

A preceptor sees an orientee omit a pending culture result during handoff. No harm occurs, and the receiving nurse obtains the information from the record. The preceptor plans to give feedback after the report.

Which feedback is most likely to improve future performance?

A) "Your handoffs are unreliable, so I will give them from now on."

B) "Be more careful next time; experienced nurses do not miss details."

C) "The pending culture was omitted; tell me what contributed and how you will include pending results next time."

D) "Nothing happened, but I will mention this in your final evaluation."

26.

Scenario

A medical-surgical unit has increasing turnover among nurses with 1 to 3 years of experience. Exit interviews mention limited growth opportunities and little input into unit decisions. The manager has funding for one focused retention initiative.

Which action is most likely to address the identified causes?

A) Create a staff-led clinical advancement project with mentoring and decision-making authority

B) Increase recruitment advertising without changing the current work environment

C) Give all remaining nurses the same retention message regardless of career goals

D) Delay action until turnover is higher than the organization-wide average

PART B - ANSWER KEY AND EXPLANATIONS

Question numbering below corresponds to the scored questions in Part A.

1	B	2	D	3	A	4	C	5	A
6	B	7	C	8	A	9	D	10	C
11	B	12	D	13	C	14	A	15	D
16	C	17	B	18	D	19	A	20	B
21	C	22	D	23	A	24	B	25	C
26	A								

1. Correct: B - Assess the patient and determine whether ambulation remains appropriate

A change in condition alters the circumstances of delegation. The RN must assess the patient and use nursing judgment before the delegated activity continues; changing the UAP does not address the patient's new symptom.

2. Correct: D - Engage staff in designing a limited trial, define outcome measures, and revise the process from feedback

Staff participation, limited testing, outcome measurement, and iterative revision build ownership while preserving accountability for improvement. Optional adoption would create an unreliable handoff process.

3. Correct: A - Ask the orientee to analyze the interaction, identify missed cues, and plan a teach-back approach

Guided reflection connects observed behavior with its effect and helps the orientee generate an actionable improvement. Taking over without timely discussion misses the learning opportunity.

4. Correct: C - Compare objective actions with practice standards and identify individual and system learning needs

Constructive peer review uses objective evidence and standards to improve practice while considering both individual and system contributors. Reputation and informal history introduce bias.

5. Correct: A - A stable patient with heart failure who needs routine oral medications and reinforcement of low-sodium teaching

This assignment fits the stated policy and involves a stable patient with predictable needs. Initial assessment, rapidly changing conditions, and IV push medication require RN-level judgment or functions excluded by the policy.

6. Correct: B - Match assignments to acuity and competencies, use the qualified float nurse, and reassess workload during the shift

Safe, cost-conscious staffing considers patient needs and staff competence, uses the available qualified resource, and includes ongoing workload review. Equal patient counts may not represent equal workload.

7. Correct: C - Assess the patient, validate relevant findings, and determine whether escalation is needed

The UAP appropriately reported a result outside the stated parameter. The RN retains responsibility for assessment, interpretation, and escalation; irrigation or fluid treatment should not be initiated by the UAP.

8. Correct: A - Provide hands-on training and practice with feedback before independent use

In ADKAR, the team has awareness and desire but lacks knowledge of how to perform the new workflow. Training and coached practice directly address that gap; reinforcement is more appropriate after the behavior is established.

9. Correct: D - Complete the required education and demonstrate competency according to organizational policy

Independent use requires verified competence with the specific device and adherence to policy. Similar-device experience or informal proximity of a coworker does not substitute for validation.

10. Correct: C - Move the discussion to a private area, clarify the patient-care facts, and facilitate a respectful solution

The charge nurse should stop public disruption, focus the discussion on objective patient-care needs, and help the nurses reach a workable resolution. Public blame can intensify conflict and damage team function.

11. Correct: B - Help the nurse define goals, assess strengths and gaps, and schedule progress reviews

Effective mentoring begins with learner-centered goals and assessment, followed by an agreed process for review. The mentor should guide rather than impose a replica of the mentor's career path.

12. Correct: D - Assist the stable patient with bathing and report any skin concerns

Bathing a stable patient is predictable and does not require nursing judgment when clear reporting expectations are provided. Assessment, evaluation, and management of acute symptoms remain RN responsibilities.

13. Correct: C - Stop the patient from entering the clean zone and route the patient through the designated decontamination process

Preventing secondary contamination protects patients, staff, and hospital operations. Registration or symptoms do not determine whether decontamination procedures can be bypassed.

14. Correct: A - Give a concise handoff naming the required follow-up and confirm that the covering RN accepts it

A direct handoff with explicit tasks and closed-loop acceptance establishes who will act during the break. Documentation or an indirect reminder alone does not confirm responsibility.

15. Correct: D - Observe the workflow, give behavior-specific feedback, and set progressive time-management goals

Direct observation allows feedback on modifiable behaviors while progressive goals develop efficiency without sacrificing medication safety. Vague pressure to work faster does not teach a safer process.

16. Correct: C - Examine the system hazards and the nurse's behavioral choices, then respond proportionately

Just culture evaluates both system design and individual behavior. The look-alike storage hazard warrants correction, while the nurse's choices should be assessed fairly rather than ignored or automatically punished.

17. Correct: B - Set a measurable objective, select relevant learning and practice activities, and evaluate performance

A complete development plan links a defined competency gap to measurable learning, purposeful practice, and evaluation. Attendance or reading alone does not demonstrate application.

18. Correct: D - Stop oral intake and assess the patient before feeding resumes

Repeated coughing during feeding may indicate impaired swallowing and aspiration risk. The UAP should stop as directed, and the RN must assess before any oral intake continues.

19. Correct: A - Clarify roles and accountability, then use brief team huddles to coordinate and update the plan

Team nursing depends on clear roles, accountability, and planned communication. Huddles create a shared plan and allow the team to respond to new information without abandoning the model prematurely.

20. Correct: B - Assign the procedure to the validated nurse and arrange competency revalidation before future independent performance

Current validation is required before independent performance of the restricted procedure. Reassignment protects the patient while revalidation restores the nurse's eligibility without falsifying records.

21. Correct: C - Review utilization against indications, address knowledge gaps, and standardize appropriate access

Reviewing actual use before intervening distinguishes justified clinical use from waste. Education and standardized access can improve stewardship without indiscriminately restricting an indicated product.

22. Correct: D - State that the situation is a safety concern and activate the chain of command if it remains unresolved

When a clearly stated concern remains unresolved and the patient may be deteriorating, assertive safety language and escalation through the chain of command support advocacy. Documentation does not replace action.

23. Correct: A - Apply a relevant practice update, evaluate its effect on care, and share learning with colleagues

Active use of professional resources to change, evaluate, and disseminate practice supports continuing competence. Membership or attendance without purposeful application provides weaker evidence of development.

24. Correct: B - Report the competing needs through the assigned chain and obtain a single prioritized direction

Incident command establishes a defined reporting structure to coordinate scarce resources and avoid competing instructions. Rank outside the assigned structure does not determine incident priorities.

25. Correct: C - "The pending culture was omitted; tell me what contributed and how you will include pending results next time."

Timely, private, behavior-specific feedback connects the omission to reflection and a future plan. Labels, comparison, or delayed vague feedback are less useful and can undermine learning.

26. Correct: A - Create a staff-led clinical advancement project with mentoring and decision-making authority

The initiative directly responds to requests for growth and influence while building professional engagement. Recruitment messaging does not correct the work-environment factors identified in exit interviews.